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Question 1

Not answered

Marked out of 5.00

Intelligent quotient

Identify the IQ scores for different degrees of learning disability according to ICD-10 classification

Mild learning disability	Choose... ▾
Moderate learning disability	Choose... ▾
Severe learning disability	Choose... ▾
Profound learning disability	Choose... ▾
Borderline intellectual functioning	Choose... ▾

Your answer is incorrect.

Explanation:

Learning disability in ICD10 is synonymous with mental retardation (DSM) and also called intellectual disability. It is defined in DSM-5 in terms of the IQ being less than 70. The presentation is in early life within the developmental period that DSM stipulates as up to 18 years of age but is not specified by ICD. It is associated with deficits in social and adaptive functioning. General intellectual functioning is determined by the use of standardized tests of intelligence.

Mild LD has IQ in the range 50-69, moderate with a range 35-49, severe in the range 20-34, and profound being below 20. Borderline intellectual functioning, according to DSM-5, is not within the diagnostic boundary of mental retardation. It refers to a full-scale IQ in the range of 71 to 84.

The correct answer is: Mild learning disability → 50-69, Moderate learning disability → 35-49, Severe learning disability → 20-34, Profound learning disability → Less than 20, Borderline intellectual functioning → 71-84

Question 2

Not answered

Marked out of 4.00

Prevalence of mental retardation

Identify the proportion of patients with learning disability represented for each level of disability described below:

Mild learning disability

Choose... ▾

Moderate learning disability

Choose... ▾

Severe learning disability

Choose... ▾

Profound learning disability

Choose... ▾

Your answer is incorrect.

Explanation: Mild mental retardation (IQ range, 50 to 70) represents approximately 85 percent of persons with mental retardation. The IQ range of moderate mental retardation is 35-50. It represents about 10 percent of persons with mental retardation. The IQ range of severe mental retardation is 20-25. It comprises about 4 percent of individuals with mental retardation. People with profound mental retardation have an IQ range below 20. It constitutes approximately 1 to 2 percent of people with mental retardation. Most individuals with profound mental retardation have some form of identifiable causes for their condition.

The correct answer is: Mild learning disability → 0.85, Moderate learning disability → 0.1, Severe learning disability → 3-4%, Profound learning disability → 1-2%

Question 3

Not answered

Marked out of 5.00

Learning disability & disorders (1)

Identify the correct condition for each of the following descriptions given below;

A 19-year-old woman with a history of epilepsy presented with skin depigmentation, white skin patches and low IQ. She has tumours in the lungs and kidneys

Choose...

A 12-year-old boy with a long face, large ears, enlarged testes and short stature. He also has some speech abnormalities

Choose...

A 6-year-old boy with small skull, oblique upward slanted palpebral fissures, protruding tongue, high arched palate and short, broad hands with a single transverse palmar crease

Choose...

A 3-year-old girl achieved normal milestones until the age of 6 months. Between 6 and 18 months, she has lost her milestones achieved previously, has had several epileptic seizures and has abnormal hand flapping and hand-wringing movements

Choose...

A 4-year-old boy with broad thumbs and great toes, the persistence of fetal finger pads, short upper lip, pouting lower lip, beaked nose and long eyelashes. He has stereotypical rocking behaviour and intolerance to loud noises

Choose...

Your answer is incorrect.

Explanation:

1) The description is suggestive of tuberous sclerosis, learning disability of various degrees can present along with intractable seizures, hamartomas of the central nervous system and depigmented skin patches (ash leaf spots) and fibromas of the nails.)

2) Fragile X syndrome is characterized by the presence of a long narrow face, large ears, short stature, macroorchidism (large testicles), midface hypoplasia, high arched palate, and macrocephaly. Speech abnormalities such as perseveration of words, echolalia, palilalia (repetition of words with increasing speed and decrescendo of volume), cluttering and narrative speech (language that is often devoid of content, yet the language structure remains intact) are reported.

3) Children with Down syndrome show hypotonia, oblique palpebral fissures, abundant neck skin, a small, flattened skull, high cheekbones and a protruding tongue. High-arched palate and a single palmar crease (simian crease) with syndactyly (webbed fingers), and altered dermatoglyphics is also noted.

4) Rett syndrome is seen exclusively in girls. There is normal development up to 6 months. Between 6-18 months, there are subtle developmental deviations. There is a development of abnormal movements like hand flapping and hand wringing, often with autistic features. There is a characteristic progression to severe disability with moderate to severe LD.

5) Rubinstein Taybi syndrome is characterized by short stature, microcephaly, broad thumb, big toes, prominent nose, wide nasal bridge, hypertelorism, beaked nose, long eyelashes, expressive language difficulties, and performance IQ being greater than verbal IQ. People with this syndrome have a friendly disposition, a propensity to self-stimulatory activities such as rocking and do not tolerate of loud noises.

The correct answer is: A 19-year-old woman with a history of epilepsy presented with skin depigmentation, white skin patches and low IQ. She has tumours in the lungs and kidneys → Tuberous sclerosis, A 12-year-old boy with a long face, large ears, enlarged testes and short stature. He also has some speech abnormalities → Fragile X syndrome, A 6-year-old boy with small skull, oblique upward slanted palpebral fissures, protruding tongue, high arched palate and short, broad hands with a single transverse palmar crease → Downs syndrome, A 3-year-old girl achieved normal milestones until the age of 6 months. Between 6 and 18 months, she has lost her milestones achieved previously, has had several epileptic seizures and has abnormal hand flapping and hand-wringing movements → Rett syndrome, A 4-year-old boy with broad thumbs and great toes, the persistence of fetal finger pads, short upper lip, pouting lower lip, beaked nose and long eyelashes. He has stereotypical rocking behaviour and intolerance to loud noises → Rubinstein- Taybi syndrome



Question 4

Not answered

Marked out of 5.00

Behavioural phenotypes in learning disability (2)

Choose one diagnosis for each of the behavioral phenotypes given below;

Stereotypic hand-wringing movements

Choose...



Inappropriate laughter and cat-like cry during infancy

Choose...



Self-injurious behaviour

Choose...



Irrepressible and affectionate, pseudomature language abilities and visuospatial motor deficits

Choose...



Attraction to water and laughing at minimal provocation

Choose...



Your answer is incorrect.

Explanation:

- 1) Rett syndrome is characterized by stereotypic hand movements (described as hand wringing), reduced interest in play in early infancy followed by autistic symptoms.
- 2) Cri du chat syndrome is characterized by inappropriate laughter and cat-cry during infancy.
- 3) Cornelia de Lange syndrome is characterized by self-injurious behaviour.
- 4) Elfin-face, irrepressible and affectionate outlook, the high prevalence of anxiety disorder, phobias and hyperacusis, in a background of surprisingly good language abilities (pseudo-mature) but visuospatial/motor deficits are seen in Williams syndrome.
- 5) Angelman syndrome is characterized by a puppet-like gait, attraction to water, happy disposition, and laughing at minimal provocation.

The correct answer is: Stereotypic hand-wringing movements → Rett syndrome, Inappropriate laughter and cat-like cry during infancy → Cri-du-chat syndrome, Self-injurious behaviour → Cornelia de Lange syndrome, Irrepressible and affectionate, pseudomature language abilities and visuospatial motor deficits → Williams syndrome, Attraction to water and laughing at minimal provocation → Angelman syndrome



Question 5

Not answered

Marked out of 5.00

Treatment methods in learning disability (1)

For the following clinical situations, identify the most appropriate treatment from the list:

This therapy may be used to help teach basic skills like toileting

Choose... ▾

This technique may be useful to deal with low self-esteem in mild LD.

Choose... ▾

Management of episodic dyscontrol and epilepsy in LD

Choose... ▾

Management of inappropriate sexual behaviours.

Choose... ▾

Management of intractable sexual offending not responding to behavioural measures

Choose... ▾

Your answer is incorrect.

Explanation:

- 1) Behavioural treatments (Shaping and chaining) may be used to help teach basic skills like toileting and establish normal behaviour patterns in people with learning disability.
- 2) Cognitive behavioural therapy could be used in people with borderline and mild learning disabilities. It may be effective in the teaching of problem-solving skills, management of anxiety disorders, dealing with issues of self-esteem and anger management.
- 3) Anticonvulsants such as carbamazepine are useful in the treatment of episodic dyscontrol and for better control of underlying epilepsy.
- 4) Behavioural treatments may also be used to alter maladaptive patterns of behaviour. It is based on operant conditioning principles and is mainly used to modify inappropriate sexual behaviours.
- 5) Anti-libidinal drugs such as cyproterone acetate and medroxyprogesterone, which reduce testosterone levels, can be utilized in the treatment of sexual offending, not responding to behavioural measures, but it is rarely used these days.

The correct answer is: This therapy may be used to help teach basic skills like toileting → Behavioural treatments, This technique may be useful to deal with low self-esteem in mild LD. → Cognitive therapy, Management of episodic dyscontrol and epilepsy in LD → Carbamazepine, Management of inappropriate sexual behaviours. → Behavioural treatments, Management of intractable sexual offending not responding to behavioural measures → Cyproterone acetate



Question 6

Not answered

Marked out of 5.00

Treatment methods in learning disability (2)

For the following clinical situations, identify the most appropriate treatment from the list:

A 43-year-old man with learning disability is paranoid about nurses on the ward and is seen responding to voices

Choose...

A 37-year-old man with learning disability is overactive and has frequent aggressive outbursts on the ward. He has a diagnosis of bipolar disorder

Choose...

A 28-year-old woman with Down's syndrome with low mood is isolating herself, doesn't engage in conversations and has poor sleep pattern.

Choose...

A 13-year-old boy with Down's syndrome is reported to be hyperactive, restless and difficult to manage. He has a reduced attention span, and occupational therapist could not engage him in any therapeutic activities.

Choose...

An 18-year-old woman with moderate level of learning disability has severe self-injurious behaviour including hand biting and biting off her lips. She does not appear depressed

Choose...

Your answer is incorrect.

Explanation:

1) Antipsychotics are used for the treatment of comorbid psychiatric disorders (e.g. schizophrenia and related psychosis) and acute behavioural disturbance. Low dose antipsychotics are also helpful to reduce stereotypies. Both oral and depot neuroleptics are frequently used in treating behavioural and psychiatric symptoms in people with learning disability. Second generation drugs such as risperidone should be tried first.

2) Lithium is useful in the treatment bipolar affective disorder and augmentation of antidepressant therapy and is also helpful in reducing aggressive outbursts. It is licensed for the 'control of aggressive behaviour or intentional self-harm'. EEG must be obtained as a baseline along with renal functions and cell counts before lithium is started. The therapeutic dose of lithium may induce seizures in those with LD and epilepsy. Patients with pre-existing EEG changes are more prone to lithium toxicity (Frith Prescribing Guidelines p.99).

3) Antidepressants are helpful for the treatment of depression, OCD, self-injurious behaviour and other anxiety disorders. SSRIs such as citalopram should be tried first.

4) This boy is exhibiting features of ADHD. Stimulants such as methylphenidate could be used for the treatment of co-morbid ADHD.

5) Opiate antagonist naltrexone may be effective in the treatment of repetitive self-injury. It may decrease self-injurious behaviour acutely, but it is less effective in the long term.

The correct answer is: A 43-year-old man with learning disability is paranoid about nurses on the ward and is seen responding to voices → Risperidone, A 37-year-old man with learning disability is overactive and has frequent aggressive outbursts on the ward. He has a diagnosis of bipolar disorder → Lithium, A 28-year-old woman with Down's syndrome with low mood is isolating herself, doesn't engage in conversations and has poor sleep pattern. → Citalopram, A 13-year-old boy with Down's syndrome is reported to be hyperactive, restless and difficult to manage. He has a reduced attention span, and occupational therapist could not engage him in any therapeutic activities. → Methylphenidate, An 18-year-old woman with moderate level of learning disability has severe self-injurious behaviour including hand biting and biting off her lips. She does not appear depressed → Naltrexone



Question 7

Not answered

Marked out of 5.00

Syndromes of Learning Disability (3)

Using the descriptions given below, identify the correct condition from the list;

This condition affects only females

Choose...

This condition is produced by mutations in a gene coding for an enzyme responsible for purine synthesis

Choose...

This condition is the commonest cause of inherited learning disability

Choose...

This condition is featured by central obesity, polydactyly and night blindness

Choose...

This condition is caused by deletion solely from the maternal genetic source

Choose...

Your answer is incorrect.

Explanation:

- 1) Rett syndrome occurs exclusively in females. Rett syndrome is characterized by stereotypic hand movements (described as hand wringing), reduced interest in play in early infancy followed by autistic symptoms.
- 2) In Lesch-Nyhan syndrome there is a defect in hypoxanthine guanine phosphoribosyl-transferase with accumulation of uric acid; It is X-linked recessive disorder.
- 3) Fragile X syndrome is the most common inherited cause of Learning Disability. This syndrome affects approximately 1 in 4425 to 6045 males and causes mental retardation in 1 in 8000 females with an X-linked dominant transmission. It accounts for 10-12% of mental retardation in males. Penetrance is low, but greater in males than females. It is due to the protective effects of the second normal X chromosome in females).

4) Laurence Moon Biedl syndrome is associated with multiple loci (11q13, 11 q21). In this condition, there is central obesity, polydactyly and mild to moderate LD. Other features include short stature, spastic paraparesis, hypogenitalism, night blindness (due to red cone atrophy), NIDDM, renal problems (diabetes insipidus and kidney failure). (Ref: Oxford handbook- pg 707).

5) In Angelman syndrome, there is deletion in 15q12 of maternal origin; 80% of cases is due to deletion of maternally derived chromosome 15.

The correct answer is: This condition affects only females → Rett syndrome, This condition is produced by mutations in a gene coding for an enzyme responsible for purine synthesis → Lesch- Nyhan syndrome, This condition is the commonest cause of inherited learning disability → Fragile X syndrome, This condition is featured by central obesity, polydactyly and night blindness → Laurence Moon Biedl syndrome, This condition is caused by deletion solely from the maternal genetic source → Angelman syndrome



Question 8

Not answered

Marked out of 5.00

Learning disability - Aetiology

Identify the proportion of cases with learning disability caused by each of the following reasons

Genetic inheritance

Choose... ▾

Pregnancy-related and perinatal problems

Choose... ▾

Acquired general medical conditions

Choose... ▾

Environmental influences

Choose... ▾

No definite etiology

Choose... ▾

Your answer is incorrect.

Explanation:

Aetiology of mental retardation: Early alterations in embryonic development (e.g. chromosomal changes such as Down syndrome & prenatal damage due to toxins such as alcohol) = 30% of cases, Pregnancy related and perinatal problems = 10%, Genetic inheritance = about 5% of cases, Acquired general medical conditions = 5%, Environmental influences = 15-20% and no clear etiology = 30-40% of cases.

The correct answer is: Genetic inheritance → 5%, Pregnancy-related and perinatal problems → 10%, Acquired general medical conditions → 5%, Environmental influences → 15-20%, No definite etiology → 30-40%

Question 9

Not answered

Marked out of 5.00

Learning disability & mental illness

Identify the estimated prevalence of mental health problems from population-based studies of adults with learning disability

Schizophrenia

Choose... ▾

Depression

Choose... ▾

Epilepsy

Choose... ▾

ADHD

Choose... ▾

Self-injurious behaviour

Choose... ▾

Your answer is incorrect.

Explanation:

1) Psychotic disorders, particularly schizophrenia, appear to be more common in those who also have a learning disability. The rate is raised threefold over the general population. Most studies report a prevalence of 3 to 3.5 percent for schizophrenia in learning disabled population.

2) The prevalence of depression is around 2-4%. Depressed mood and vegetative symptoms are the most striking symptoms, even though complaints of depression are not always expressed. A depressive mood is not verbalized, particularly among individuals at a lower intelligence level, but may well be observable.

3) Epilepsy is more prevalent and harder to diagnose and treat in people with mental retardation than in those with normal intellect. Population-based studies have revealed that mental retardation occurs in at least 30 to 40 percent of individuals with epilepsy while the prevalence of epilepsy in the mentally retarded population is about 20 to 25 percent. It is higher in severe (30-50%) than mild (15-20%) LD.

4) Autistic spectrum disorder and ADHD is increasingly common with increasing severity of LD. It is often a prominent feature in children with LD (up to 20%). Stimulants may help in persons with mild LD with apparent symptoms, but has no clear efficacy in severe-profound LD.

5) The self-injurious behaviour occurs in approximately 20% of adults with learning disability and up to 50% in those requiring institutional care. The self-injurious behaviour occurs more often among persons with moderate and severe mental retardation ($IQ < 50$). This type of behaviour occurs most frequently between the ages of 10 and 30, with a peak between 15 and 20 years of age.

The correct answer is: Schizophrenia → 3-3.5%, Depression → 4%, Epilepsy → 20-25%, ADHD → 15-20%, Self-injurious behaviour → 15-20%



Question 10

Not answered

Marked out of 5.00

Learning disability & epilepsy

Identify the frequency of epilepsy in each of the following disorders;

Down's syndrome	Choose... ▾
Fragile X syndrome	Choose... ▾
Rett syndrome	Choose... ▾
Angelman syndrome	Choose... ▾
Tuberous sclerosis	Choose... ▾

Your answer is incorrect.

Explanation:

- 1) Epilepsy is commonly associated with numerous LD syndromes e.g. Down's syndrome (5-10%), fragile X (25%), Angelman syndrome (90%) and Rett syndrome (90%). In people with Down syndrome the frequency of epilepsy is 5 to 10 percent. There is an age-related bimodal distribution with about 40 percent of seizures starting before the age of 1 year and another 40 percent starting after the third decade.
- 2) Roughly 25 percent of individuals with the fragile X syndrome have epileptic seizures that are usually infrequent, mild, easily controlled and typically disappear in adolescence, as in benign Rolandic epilepsy.
- 3) In Rett syndrome epilepsy affects up to 90 percent of patients. Seizures are usually benign during the early years of life.
- 4) In Angelman syndrome epilepsy is present in more than 90 percent of the affected individuals.
- 5) In tuberous sclerosis, Seizures are very common (90% of cases) and often the initial presentation in an infant with 'salaam attacks' (infantile spasms) being characteristic.

The correct answer is: Down's syndrome → 5-10%, Fragile X syndrome → 20-25%, Rett syndrome → 90%, Angelman syndrome → 90%, Tuberous sclerosis → 90%



Question 11

Not answered

Marked out of 5.00

Clinical genetics

Choose one most appropriate option for each of the following descriptions

This occurs exclusively in females and the gene associated is MECP2 at Xq28

Choose...

The penetrance is greater in males than females. The gene sequence has been cloned and designated as FMR-1

Choose...

Congenital disorder caused by a microdeletion of chromosome 22, associated with learning disability and schizophrenia

Choose...

Autosomal dominant condition with NF1 gene 17q11.2 involved. The gene product is thought to be a tumour suppressor gene

Choose...

X-linked recessive condition due to a mutation in HGPRT gene

Choose...

Your answer is incorrect.

Explanation:

- 1) Rett syndrome occurs only in females. Although the etiology is unknown, it is thought to be associated with MECP2 at Xq28.
- 2) The most common inherited cause of learning disability is fragile X syndrome. The penetrance is greater in males than females. The fragile X mental retardation 1 (FMR-1) gene is responsible for fragile X syndrome caused by abnormal trinucleotide CGG repeat at a fragile site on the X chromosome (Xq27.3).
- 3) Velocardiofacial syndrome is also known as DiGeorge's syndrome. It is a congenital disorder caused by a microdeletion of chromosome 22.
- 4) Neurofibromatosis: Autosomal dominant condition with 50% new mutations. More than 90% patients have paternal NF1 allele mutated. NF1 gene is located at 17q11.2. The gene product is neurofibromin, thought to be tumour suppressor gene.

5) Lesch-Nyhan syndrome is X-linked recessive condition due to a mutation in HGPRT gene on the short arm of chromosome Xq26-27. There is a defect in hypoxanthine guanine phosphoribosyl-transferase with accumulation of uric acid; Note that a partial HGPRT deficiency results in recurrent gout.

The correct answer is: This occurs exclusively in females and the gene associated is MECP2 at Xq28 → Rett's syndrome, The penetrance is greater in males than females. The gene sequence has been cloned and designated as FMR-1 → Fragile X syndrome, Congenital disorder caused by a microdeletion of chromosome 22, associated with learning disability and schizophrenia → Velocardiofacial syndrome, Autosomal dominant condition with NFI gene 17q11.2 involved. The gene product is thought to be a tumour suppressor gene → Neurofibromatosis, X-linked recessive condition due to a mutation in HGPRT gene → Lesch-Nyhan syndrome



Question 12

Not answered

Marked out of 5.00

Location of genetic deletions

Identify the site of genetic deletion in each of the following disorders:

Angelman syndrome

Choose... ▾

Criduchat syndrome

Choose... ▾

Velocardiofacial syndrome

Choose... ▾

Smith Magenis syndrome

Choose... ▾

Williams syndrome

Choose... ▾

Your answer is incorrect.

Explanation:

1) Angelman syndrome is caused due to deletion in 15q12 of maternal origin; 80% due to deletion of maternally derived chromosome 15. In Prader-Willi syndrome there is deletion in 15q12 of paternal origin; 75% deletion of paternally derived chromosome 15.

2) Criduchat syndrome is due to a partial deletion 5p15.2

3) Velocardiofacial syndrome is associated with microdeletion of chromosome 22q11.2; 90% arise denovo, with 10% having an affected parent.

4) Smith-Magenis syndrome is associated with a complete or partial deletion of 17p11.2

5) Williams syndrome is also known as 'hypercalcemia with supra valvular aortic stenosis'. Clinical features include short stature, growth retardation, unusual facial features including the broad forehead, elfin-like facies, hoarse voice, and cardiovascular abnormalities (supra valvular aortic stenosis). Verbal skills often better than the motor and visual spatial skills.

The correct answer is: Angelman syndrome → 15, Cri-du-chat syndrome → 5, Velocardiofacial syndrome → 22, Smith-Magenis syndrome → 17, Williams syndrome → 7



Question 13

Not answered

Marked out of 5.00

Behavioural Phenotypes (2)

Choose one diagnosis for each of the behavioral phenotypes given below:

Shy, gaze avoidance and social anxiety

Choose... ▾

Compulsively severely self-mutilating self-injurious behaviours such as biting of lips, inside of mouth and fingers

Choose... ▾

Insatiable appetite, hyperphagia and obesity

Choose... ▾

Stereotypies including self-hugging, severe self-injurious behaviours like head banging

Choose... ▾

Oppositional and attention deficit problems in childhood

Choose... ▾

Your answer is incorrect.

Explanation:

- 1) Fragile X syndrome is characterized by shyness, gaze avoidance, social anxiety (more characteristic), anxious inattentive, hyperactive, mimicking schizotypal disorder (males). 5-10% have an autistic disorder but most responsive to social cues and form attachments.
- 2) Lesch-Nyhan syndrome is associated with compulsive and severe mutilating self-injurious behaviour.
- 3) Around 80% of patients with Prader-Willi syndrome show hyperphagia. Affected individuals have an almost insatiable appetite leading to stealing of food and consume unpalatable food sometimes.
- 4) Individuals affected by Smith-Magenis syndrome have flattened midface, abnormally shaped upper lip, short hands and feet, abnormally placed ears often with recurring otitis media, protruding tongue, high arched palate, and moderate to severe mental retardation.
- 5) Oppositional defiance and attention deficit problems in childhood are behavioural phenotypes seen in some children with Down's syndrome

The correct answer is: Shy, gaze avoidance and social anxiety → Fragile X syndrome, Compulsively severely self-mutilating self-injurious behaviours such as biting of lips, inside of mouth and fingers → Lesch-Nyhan syndrome, Insatiable appetite, hyperphagia and obesity → Prader-Willi syndrome, Stereotypies including self-hugging, severe self-injurious behaviours like head banging → Smith-Magenis syndrome, Oppositional and attention deficit problems in childhood → Down's syndrome



Question 14

Not answered

Marked out of 6.00

Behavioural phenotypes in Learning disability (3)

Choose one diagnosis for each of the behavioral phenotypes given below

Schizophrenia like psychotic disorder	Choose... ▾
Prominent sleep disorder	Choose... ▾
Early onset dementia in adulthood	Choose... ▾
Social anxiety disorder	Choose... ▾
Obsessive-compulsive behaviour	Choose... ▾
Anxiety disorder and phobias	Choose... ▾

Your answer is incorrect.

Explanation:

- 1) Velocardiofacial syndrome is also called as Di George syndrome, and this condition is associated with schizophrenia.
- 2) San Filippo syndrome is related to a prominent sleep disorder.
- 3) Down syndrome is associated with early onset dementia in adulthood.
- 4) Fragile X syndrome is associated with the social anxiety disorder.
- 5) Prader-Willi syndrome is associated with OCD (Levitas and Read, 1998).
- 6) Williams syndrome is related to anxiety disorder and phobias (Einfield et al. 2001)

The correct answer is: Schizophrenia like psychotic disorder → Di George syndrome, Prominent sleep disorder → San Filippo syndrome, Early onset dementia in adulthood → Down syndrome, Social anxiety disorder → Fragile X syndrome, Obsessive-compulsive behaviour →

Prader-Willi syndrome, Anxiety disorder and phobias → Williams syndrome

Question 15

Not answered

Marked out of 2.00

Genetic causes of LD

With the information given below, identify the most appropriate learning disability syndrome:

A 23-year-old man with a mild learning disability, marfanoid stature and visual problems. On examination, he had chest deformity and dislocated lens of the eye.

Choose...

A 7-year-old boy with dwarfism, claw hand, joint stiffness and hepatosplenomegaly. He has a severe learning disability.

Choose...

Your answer is incorrect.

Explanation:

1) Homocystinuria is inherited as an autosomal recessive trait. Homocystinuria has several common features with Marfan syndrome. In Marfan syndrome, the joints tend to be "loose," but in homocystinuria the joints tend to be "tight". The symptoms may occur as mildly delayed development or in some cases failure to thrive. The person may develop increasing visual problems that may lead to the diagnosis of this condition. Chest deformities (pectus excavatum, pectus carinatum), high arched feet, tall & thin build, long limbs, mild mental retardation, near sightedness, dislocated lens of the eye are other associated features.

2) San Filippo disease is related to disorders of the breakdown of heparan sulphate. Clinical features include severe LD, claw hand, dwarfism, hypertrichosis, hearing loss, hepatosplenomegaly, biconvex lumbar vertebrae and joint stiffness.

The correct answer is: A 23-year-old man with a mild learning disability, marfanoid stature and visual problems. On examination, he had chest deformity and dislocated lens of the eye. → Homocystinuria, A 7-year-old boy with dwarfism, claw hand, joint stiffness and hepatosplenomegaly. He has a severe learning disability. → Sanfilippo disease.

Question 16

Not answered

Marked out of 5.00

Behavioural phenotypes in Learning Disability

Choose one diagnosis for each of the behavioral phenotypes given below;

Stereotypic hand movements described as hand washing or hand wringing

Choose... ▾

Inappropriate laughter and cat-like cry during infancy

Choose... ▾

Self-injurious behaviour

Choose... ▾

Irrepressible and affectionate, Pseudomature language abilities; visuospatial motor deficits

Choose... ▾

Attraction to water and laughing at minimal provocation

Choose... ▾

Your answer is incorrect.

Explanation:

- 1) Rett's syndrome- Stereotypic hand movements (described as hand washing or hand-wringing), reduced interest in play in early infancy followed by autistic-like symptoms.
- 2) Cri du chat syndrome- Inappropriate laughter, cat-cry during infancy.
- 3) A self-injurious behaviour is one of the behavioural phenotypes seen in Cornelia de Lange syndrome.
- 4) William syndrome- elfin-face, irrepressible and affectionate, anxiety disorder, phobias, hyperacusis, good language abilities (pseudo-mature), Visuospatial/motor deficits
- 5) Angelman syndrome- Puppet like gait, attraction to water, happy disposition, laughing at minimal provocation

The correct answer is: Stereotypic hand movements described as hand washing or hand wringing → Rett's syndrome, Inappropriate laughter and cat-like cry during infancy → Criduchat syndrome, Self-injurious behaviour → Cornelia De Lange syndrome, Irrepressible and

affectionate, Pseudomature language abilities; visuospatial motor deficits → Williams syndrome, Attraction to water and laughing at minimal provocation → Angelman syndrome



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